

Insurance Prior Authorization Procedures

Sunrise Medical Center

Document Title: Insurance Prior Authorization Procedures

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Department: Patient Financial Services / Utilization Review

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Distribution: All Sunrise Medical Center Staff

1. Purpose

This procedure defines the process for obtaining prior authorization from commercial and government insurance payers before scheduled procedures, imaging studies, and elective admissions, in order to prevent claim denials and unexpected patient financial liability.

2. Services Requiring Prior Authorization

The Prior Authorization team maintains a payer-specific matrix, updated monthly, identifying which CPT and HCPCS codes require authorization for each contracted payer. Scheduling staff must check this matrix before booking any elective MRI, CT with contrast, elective surgery, durable medical equipment, or planned inpatient admission.

3. Authorization Request Workflow

Requests are submitted through the payer portal whenever available, or by phone/fax when portal submission is not supported, at least 5 business days prior to the scheduled service to allow for payer turnaround time and potential appeal of an initial denial.

- 1 Verify the patient's active insurance eligibility and benefits for the specific service.
- 2 Confirm the ordering provider has documented medical necessity meeting the payer's clinical criteria.
- 3 Submit the authorization request with supporting clinical documentation attached.
- 4 Log the submission, confirmation number, and expected turnaround date in the Prior Authorization Tracking System.
- 5 Follow up if no response is received within the payer's stated turnaround window, escalating to a peer-to-peer review request if needed.

4. Denials and Peer-to-Peer Review

If a prior authorization is denied, the ordering physician is notified immediately and offered the opportunity to request a peer-to-peer review with the payer's medical director. Elective services must not proceed until authorization is obtained or the patient has been informed in writing of their potential financial liability and has consented via the Advance Beneficiary Notice or payer-equivalent form.

5. Urgent and Emergent Exceptions

Emergency services are never delayed for prior authorization purposes under EMTALA and hospital policy. For urgent but non-emergent services, retrospective (post-service) authorization requests are submitted within 24–48 hours, and the case is flagged for expedited payer review citing clinical urgency.

6. Patient Communication

Patients are informed of authorization status prior to their appointment whenever the process timeline allows. If authorization is still pending on the day of service for a non-urgent procedure, the scheduling team must discuss rescheduling options with the patient rather than proceeding with an unauthorized service that could result in a large out-of-pocket bill.

7. Payer Turnaround Time Reference

Payer Type	Standard Turnaround	Expedited Turnaround
Medicare Advantage	14 calendar days	72 hours
Commercial PPO	5–10 business days	24–72 hours
Medicaid Managed Care	14 calendar days	72 hours
Workers' Compensation	Varies by state, typically 5–14 days	N/A

8. Related Documents

SMC-FIN-044 Medicare and Medicaid Billing Guidelines; SMC-UR-003 Utilization Review and Two-Midnight Rule Procedure; SMC-FIN-052 Patient Financial Counseling and ABN Procedure.

Appendix A: Frequently Asked Questions

Q: What if a payer's portal is down and a submission deadline is approaching? A: Submit via the payer's backup fax or phone line and document the portal outage with a timestamp and screenshot if possible, to support a timeliness appeal if needed.

Q: Can a patient be billed if the hospital failed to obtain a required prior authorization? A: This is handled case-by-case with Patient Financial Services leadership; hospital-caused authorization failures are generally not passed on to the patient as a surprise bill.

Q: How is an authorization handled if the ordering physician changes the planned procedure after approval? A: A new or amended authorization request must be submitted reflecting the updated CPT code, since payers authorize specific coded services, not general treatment plans.

Appendix B: Common Scenarios

Scenario — Same-day add-on procedure during a scheduled surgery: If an unexpected additional procedure is required intraoperatively, retrospective authorization is submitted within 24-48 hours with an operative note explaining the intraoperative finding that necessitated the change.

Scenario — Patient with dual Medicare and Medicaid coverage: Both payers' authorization requirements must be checked independently, as one program's approval does not guarantee the other's, and coordination of benefits rules determine billing order.

Revision History

Version	Date	Description
1.0	Jul 12, 2021	Initial release
1.3	Sep 9, 2025	Updated payer turnaround matrix and peer-to-peer process